

I: The first question is about familiarity with AI. How familiar are you with AI technologies in general?

P: I would say average. I am familiar with it. Also for my job, we don't use it, definitely because we want to counteract for all the platforms where they do use AI. So we try as much as possible to not use any of it and to really value the personal connection.

I: Could you also score this on a range of 1 to 10 on how familiar you think you are?

P: Just in my job or outside of it?

I: Just in general.

P: I think there are so many... stuff out there at the moment that you cannot be very high for the amount of time that I put in it, but at least a six or seven for my kind of day where I use it for.

I: So your organization hasn't introduced any AI tools, right?

P: Not for the work that we're doing, but we do use AI for the research that we do. So for example, to scan all the data that we have to make conclusions out of that. We do use AI for that, just because it goes quicker, but not for the actual work.

I: Is that a part of your organization's choices? Did they tell you that you should be doing this?

P: For our work, we do really tell to actually not use it. Not at all, not a single bit. To really have it as personal as possible. And also for every person who's working in our organization to have it as like an individual person. So we are actually recognizable by the person that you are. And so everything is the same with AI.

I: But then the research part, you're allowed to use AI.

P: I'm not on the research part myself, but I do know that they do use it.

I: What are your thoughts on AI being used in diagnostic or assessment processes?

P: I think I'm not as standard with this as the normal psychologist because I moved away fully from diagnosis. So where I work, we don't use diagnosis at all. We don't use anything around diagnosing people or treating a diagnosis. We only see the person behind the diagnosis. So I would say for the work that I do, where I work, I don't like it, because I don't like diagnosis in general, because it's just way too unclear for what's actually going on with the person. But I would say in general, if you would diagnose, because I also have been working for a while as a psychologist, in a previous assessment role, and there I was only diagnosing the whole day long, like every 20 minutes I had another one. For that stuff you can easily automate it, because I was just asking the same questions over and over again. So that depends on the purpose. For where I work, absolutely not. For all the other stuff, yeah, it can make your life so much easier.

I: So why are you not diagnosing as much? Because the person has very broad problems? Like psychosis, I think there's a very wide range of things you can have, right?

P: And also because we don't want to have people identified with their diagnosis. And most of the time when you have a diagnosis, then just only the diagnosis is being treated. So it's like for one simple diagnosis, they have just this protocol they follow instead of looking at the person and what the person actually needs. And that's mostly why we fully moved away from it. So we don't do anything with it.

I: And so what do you do for treatment? Because if you're not diagnosing, there is not like a set way of this is what you should be doing. So what do you choose to do?

P: We don't have the strict programs, the protocols that you really have to follow step by step. We do have kind of a way of working. And also we have different kind of chats. We have just like a listening ear chat. We have a recovery focused chat. And we have a five times project, which cannot be more than five times because then it would be official treatment, which we're not doing. And then we're mostly working with the developmental stages of Erikson, and attachment styles, for example, coping mechanisms. So we mostly move away from, forget your label, and we move towards, who are you as a person? How did you come to this point? What are your strengths, your vulnerabilities and what do you need now to be all right?

I: So this is short term, not really treatment, you said?

P: Officially, it's not allowed to be treatment, but it's like short term.

I: Yeah, because also the diagnosis is usually needed for insurance and stuff.

P: Yeah, and this is free.

I: So this is just free. And how is it paid then?

P: By [HEALTHCARE ORGANIZATION]. We are a project within that organization. And staff are paid for their hours and shifts within the organization.

I: So you think it might not be useful for you, but you think in general there might be some people that could use it for intake processes, right. Have you considered AI for preliminary screening?

P: No. And will also not consider.

I: What about summarizing patient information?

P: Could be cool, but also we are only working with anonymous patients. So everyone is anonymous. We can summarize whatever they talk about, but also they can just download the chat at the end and then they can read it back, including the whole interaction. And next to that,

we cannot really summarize stuff because it's anonymous. So also between chats, we are not allowed to make summaries between chats. This might be too different.

I: And so how does it work if it's anonymous? How do you keep track of the five sessions you have?

P: Because the appointments we make and they can have an appointment code to come in and then they just can come under an anonymous name. And just in the chat, we make an appointment for the next one. And then when I see them in the waiting room, I will just pick them up and then they end up with the right person.

I: How do you view the role of AI in tracking patient progress over time?

P: I think for our thing, not too necessary, but I think in general it can be nice. Just because I noticed for my internship, I needed to do a lot of questionnaires and I was absolutely done with it. But every person after the third session and fifth session, so soon I was done. So I think for that stuff, it's going to be super nice and helpful.

I: So in your current situation, you don't really have that much tracking and monitoring because it's very short term, right? But you've previously already experienced some normal tracking in a more basic scenario, right? How many times did you do the monitoring and tracking?

P: I did an internship of a year, I worked for personality disorders. And for that, I did the research for, I don't know, eight months, ten months. And every week I had 10 or 15 people, I guess, which took two hours to go through all the questions.

I: And it's all just the same type of questions?

P: Yeah. It depends a bit on the kind of personal development or personal disorder they have, but yeah, in general it's the same. Give a score between this and this, give a score between this and this.

I: What concerns, if any, would you have about AI tracking patient progress?

P: That's a good one. At the moment, not too much, but also because I'm not using it, I'm not too familiar with it. I would say that people miss the personal connection with humans. As also all the questions were already not nice for anyone to do. And then we could have some kind of interaction, some kind of fun, some kind of making the situation lighter because I'm actually human instead of having everything only with AI. And I think definitely in our healthcare and with the mental health problems, the personal connection is way more needed than ever before. Also in our society where everyone's already living so lonely, so in isolation. So I would say mostly the human factor in it.

I: How do you feel about AI generating drafts or reports or assisting with scheduling and reminders?

P: Yeah, that's perfect. Because I think that's not the nicest focus where people who are trying to be a psychologist can get their attention to. Like, it's not their strength, probably. And I think they can use their valuable time more valuably.

I: So would you trust AI-generated reports or would you need to review everything?

P: That depends on what it is reporting. In general, I would say yes. Sometimes when you see, like, summaries from Zoom, that's absolutely shit. Then no. So it depends a little bit on what's been talked about and how well it's developed.

I: So it's dependent on what type of AI it is. If it shows that it is quite good at making summarizations, then maybe you're less inclined to review it. So how much time could this save you?

P: From my job not too much because I'm not using any of it. In general I guess it could do a lot, because most of the time, I'm just thinking, when I was doing my internship I saw five to seven people per day but I was still there for nine hours. So I think an hour per day at least.

I: So what concerns would you have about accuracy or liability? That's kind of just dependent on how well it works, right. And you're currently not using it. So would this change how you document your work?

P: Probably not.

I: What are your thoughts on using AI for psychoeducation or therapeutic exercises between sessions?

P: I would say it can be very helpful, but mostly as an extra support where humans don't have enough time. And also I used to work for an online department of this organization. And there we also used it a lot already, a little bit in the case of homework or sending reminders daily to do your self-compassion exercises or whatever. So I would say for that it's nice just because you have more tools while you don't have more time yourself.

I: So what do you currently do in between the sessions?

P: Not too much because it's anonymous. So we can also not reach out to them. We just make agreements on, for example, homework or stuff that they will work on in between. And then we check in after. But it's still always a little bit of a gap where you mostly lose people, mostly the depressed, low energy people. And I know from my internship that it should work very well to get them the daily notification or a reminder or a personalized one.

I: So what if the AI provides information that contradicts your treatment approach?

P: That's fascinating. Or the AI is wrong, or your approach is wrong, or there's a method discrepancy, which can also be interesting to evaluate with those people, what aligns more, what does not align more. So I would say it's a very interesting situation to approach with the

client, but also it can be kind of shit when it really goes against you. And when it really goes against all of them, because I know a lot of people do trust the AIs more than the humans, but I also can recognize it with myself. Yeah, but he says it, so I think it's true. And there's the tricky thing that they will stop thinking for themselves. And then it can harm the therapeutic relationship.

I: Would you worry about clients relying too much on AI between sessions?

P: Yeah, a bit. Because for example, the typical chat that they know will never go against you. So whatever they're saying, it will be confirmed, while most of the therapy is to not confirm their current pattern, but to go against it. And this is a tricky thing.

I: But this is also very specific for your client population, right?

P: Partly, but I think there'll be a lot of cases where you go against the current patterns, the current beliefs.

I: How would you monitor what the AI is telling your clients?

P: Probably just by letting it make reports and then still reading it all, but it also kind of misses the point then if you still need to read everything. So how to monitor that, yeah, you can ask clients, but also then maybe the therapeutic relationship is already shaking. Or it should be AI that you very well know is following a program and will not move away from it. But it would not be the nicest thing. So actually I never thought about that.

I: What risks or concerns do you have about using AI in clinical work?

P: Lacking a personal connection. Outsourcing responsibility, that they outsource it to an AI instead of taking responsibility for their own life. Having everything confirmed instead of being challenged. Too much standardizing in the programs instead of adapting it really to this person and what this person needs. And also missing the reading in between the lines. Because most of the time people say a lot, but there's actually something else that's going on that a person can feel or can notice. But the AI who takes everything literally might not. Something like that.

I: So what concerns do you have about transparency or explainability of AI decisions?

P: What?

I: Transparency and explainability.

P: What worries I have around that? I think that's mostly from the side of the clients, or from the side of the people, that they might not be aware of it, that it's an AI rather than another person, and they will treat it as a person, which can also harm their whole personal view of people, or improve it, depending on AI. Next to that, I don't have too much worries.

I: And how about patient privacy and data security?

P: Depends on the platform. For us already, everything is anonymous. So if there would be an AI on this, it would still be very anonymous. But I would say in general, it is a tricky thing because once it's in any AI, it's somewhere online. Or it's somewhere. And then it's always more tricky than when it's nowhere.

I: Have you thought about issues like bias in AI systems? This is kind of what you mentioned with the standardizing, right, and the non-personal connection.

P: Yeah. There can always be a bias in it, also depending on who programmed the AI and what the purpose is of the AI. The AI is always programmed for a specific purpose, but maybe that's not the same as the purpose of where the client is going for.

I: Any other limitations you see that you want to mention?

P: Does it make life a bit boring? But next to that, no, not too much.

I: Your life or in general?

P: I would say in general. Like where's the human and where's the connection if all the people are being removed.

I: How do you think clients respond to AI being used in their treatment?

P: Depends very much on the client. Because what I know with a lot of people around psychosis or schizophrenia, they are already very distrustful of people in general, they don't trust anyone. So then I would say for them, they already come into the chat with us like, are you a real person or not, because they already don't trust anything or anyone. And I do think that with that specific population, it will absolutely make everything worse. When they're already convinced that everyone's watching them, they're already monitored everywhere, if they would then talk with the AI.

I: So now you also get the question, like, are you a real person?

P: Yeah, we get it a lot. But I think for other people, it can also be nice. For example, clients who are very much in fear, who already have a lot of anxiety around people, for them it can help to start with AI. I would say it depends on the client.

I: So how do you tackle that? How do you convince someone that you are a real person talking to them and not an AI?

P: Sometimes easy. When they say, are you a real human? And they're just like, yeah, luckily I am, with a joke or something. I still don't believe it. And then they try to ask us something. But then depending on our answer, they still don't trust us. So it's always a little bit of a thing. But also it's mentioned everywhere on our websites, because we only work with real people with personal experience.

I: That must be very weird, to try and convince someone you're a real person.

P: It's actually super strange, definitely when they don't believe you.

I: It's also very hard, like how do you even test that? You can ask them, can you tell me about the news yesterday or something, but I could probably also find that, so it's really hard to check.

P: So normally we try to do it with personal stories, because everyone who is working with us also has their own experience in mental health. So then we can relate with, oh yeah, what I noticed when I was in therapy. But it's a new challenge that we didn't have four years back.

I: So is that also increasing now?

P: yes

I: That makes sense. Can you also give a score for this? So how do you think clients respond to AI being used in treatment, on a score of one to ten, one being they would definitely not want it, and ten being they would really like it?

P: Depends on the type of clients, but I would say a four, in general, mostly taking my kind of clients into account.

I: How might this affect the therapeutic alliance?

P: If you use AI?

I: Yeah, introducing AI into treatment.

P: I think a lot of people already don't feel like they are being seen, being heard, being taken seriously. I think if you would just say, hey, I absolutely don't care, here's AI, it will not really help. So I would say then they would go into desperation sooner, which is already a challenge for a lot of people. Because I can imagine that it feels like you absolutely don't care, when they try to apply for some kind of mental health support, they are on a waiting list for like eight months to a year, and then it's like, hey, cool, you're here, here's AI. But maybe that's just one scenario that I'm seeing now. Probably it can also be good. But that's the first reaction where I'm like, hmm.

I: So in your current organization, they're also on a waiting list for eight months?

P: No, we don't do that. But I would say in general, yeah, there's a lot of waiting lists, but we don't have it.

I: So with you, is there any waiting or is it you can just get in tomorrow?

P: Depends on how many people are available and how many people are in the waiting queue. Because we just have a waiting queue for where they can enter. If they don't have an appointment, they might be waiting until someone is available again. And the chats can be from

40 minutes to two and a half hours, three hours per person. So it can then take a while, but there's not really longer waiting than a day.

I: Do you think clients will be more or less engaged with AI-supported treatment?

P: I would say less. Because you already know that another person is reviewing it, so also all the guilt around, oh, I have to do this otherwise something will happen, will drop. Because I would not take it seriously when I have to do stuff because an AI is telling me to.

I: Which aspects of your work should remain strictly human? And what would you need to feel confident about using AI safely in practice?

P: The actual contact, the treatment needs to stay human. What would make me more confident to use it? It's for sure to try it for a while and to really give specific tasks to it. Like all the administration stuff, all the behind the scenes stuff, I would trust it more to outsource than the responsibility over a living human being.

I: So why do you think these aspects need to stay human? You kind of already mentioned it in the interview, but maybe you could summarize.

P: Just because I became a psychologist to actually help people and I want to help people, and I think people deserve to be helped by a human that also values human connection in this whole world where isolation is already so massive.

I: I think that's it. Do you have anything else to add?

P: Not really.